

CO-DESIGNING AND OPTIMISING AN ONLINE NUTRITION EDUCATION PROGRAM FOR PEOPLE WITH MULTIPLE SCLEROSIS

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INTRODUCTION

People with multiple sclerosis (MS) frequently seek dietary advice; however, online nutrition information is often conflicting and not evidence-based. In people with MS, co-design is particularly important due to the variability of symptoms, the complexity of dietary information available, and the strong interest in lifestyle management strategies [1]. To address this, our team developed Eating Well with MS, a co-designed 6-week digital program providing evidence-based nutrition education and behaviour change support for people with MS in Australia [2].

A feasibility study (n = 70) showed that the Eating Well with MS was acceptable and feasible, with preliminary data suggesting improvements in diet quality, food literacy, and nutrition literacy [2]. Building on these findings, this study aimed to further co-design, optimise, and digitally redevelop Eating Well with MS to strengthen its content, behaviour change support, accessibility, and delivery in preparation for evaluation in a large-scale randomised controlled trial (RCT).

METHODS

A multiphase participatory design process involving people with MS and carers was undertaken between 2023–2026 to inform redevelopment of Eating Well with MS within Platform O, a digital delivery platform with interactive features including user tracking, videos, audios, discussion boards, and other digital resources. The co-design process included: a) semi-structured interviews with prior program participants (n = 16) [3]; b) stakeholder (e.g., clinicians and MS dietitians) focus groups (n = 31) and online workshops (n = 9); and c) three Lived Experience Advisory Panel meetings (n = 9). Participants and stakeholders provided feedback on program content, relevance to MS-related needs, behaviour change support, usability, accessibility, digital delivery, and recruitment pathways.

RESULTS AND DISCUSSION

Following this process, key refinements included: a) restructuring of the program content and introduction of a 3-month refresher module, incorporating additional goal-setting and fatigue planning activities to strengthen longer-term behaviour change; b) updated evidence and resources to reflect the latest research; c) new content on different life stages such as pregnancy and menopause; d) integration of audiovisual accessibility features, including text-to-audio narration; e) improved navigation, delivery, and engagement tracking via Platform O, enabling automated module release, embedded analytics, and participant dashboard tracking; and f) expanded recruitment reach and pathways informed by stakeholder consultation.

CONCLUSIONS

The Eating Well with MS program was further co-designed, optimised, and digitally redeveloped to support accessible, larger-scale digital delivery and RCT evaluation among people with MS in Australia. Integration of stakeholder and lived-experience perspectives enhanced the program's relevance, accessibility, and behavioural support prior to RCT implementation.

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